

CLINICAL INTELLIGENCE TREND REPORT

RED: Provider Review Recommended

Patient ID: JBPeriod: July 2 to September 30, 2024Report Date: October 1, 2024Coverage: Bed: 1772/2174h (81.5%)Chair: 1474/2174h (67.8%)

Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Clinical Alerting Thresholds

Very Low HR < 40 bpm

Very High HR > 110 bpm

Low HR < 45 bpm

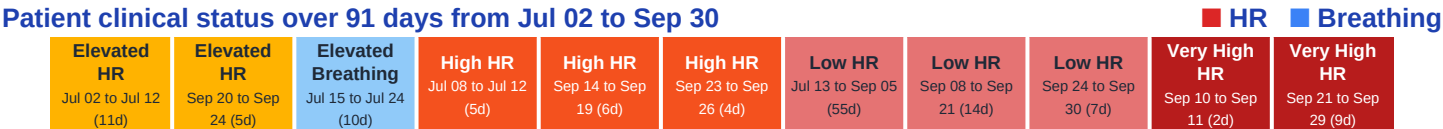
Elevated Breathing > 24 brpm

Elevated HR > 95 bpm

High Breathing > 30 brpm

High HR > 100 bpm

Very High Breathing > 40 brpm



Last 24 Hours

Last 24 Hours: RED			30-Day Tier: RED		
Vital	Avg	Min	Max		
Heart Rate	78 bpm	40 bpm	145 bpm		
Breathing	15 brpm	11 brpm	31 brpm		

Last 24 hours: three low heart rate episodes in the evening; one very high heart rate episode in the evening.

Episodic Events (last 24h)

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
Sep 29	8 PM to 6 AM	14h	4	Elevated Breathing	15	11	31	Check SpO2 and lung sounds; assess for fluid overload or early infection
Sep 29	9 PM to 11 PM	4h	3	Low Heart Rate (brief)	44	40	50	Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
Sep 29	8 PM to 6 AM	10h	1	Very High Heart Rate (brief)	139	71	145	Check temp, hydration, pain; review for arrhythmia or infection

291 episodic events Detected, spanning 26 days total. Conditions: Elevated Breathing, Elevated Heart Rate, High Breathing, High Heart Rate, Low Heart Rate, Very High Heart Rate. Priority grade: High

Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
291	624h	Low Heart Rate	3	100%

Major Findings: Sustained low HR (avg 43, min 40)

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
Sep 10	7 AM to 1 PM	35h	5	Very High Heart Rate	141	74	148	Check temp, hydration, pain; review for arrhythmia or infection
Sep 21	3 AM to 8 AM	66h	9	Very High Heart Rate	137	68	145	Check temp, hydration, pain; review for arrhythmia or infection
Jul 08	12 AM to 1 AM	7h	6	High Heart Rate	104	45	127	Check temp, hydration, pain; review for arrhythmia or infection
Sep 14	5 AM to 6 AM	26h	14	High Heart Rate	105	51	130	Check temp, hydration, pain; review for arrhythmia or infection
Sep 23	6 PM to 8 PM	7h	6	High Heart Rate	105	55	145	Check temp, hydration, pain; review for arrhythmia or infection
Jul 13	10 PM to 11 PM	339h	155	Low Heart Rate	43	40	74	Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
Jul 07	8 AM to 10 AM	20h	12	High Breathing (brief)	33	15	55	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF

+ 5 additional condition(s) across Elevated Breathing, Elevated Heart Rate, Low Heart Rate; details in monitoring data.

The P5 to P95 heart rate spread was 54 bpm (43 to 97), indicating significant cardiac variability.

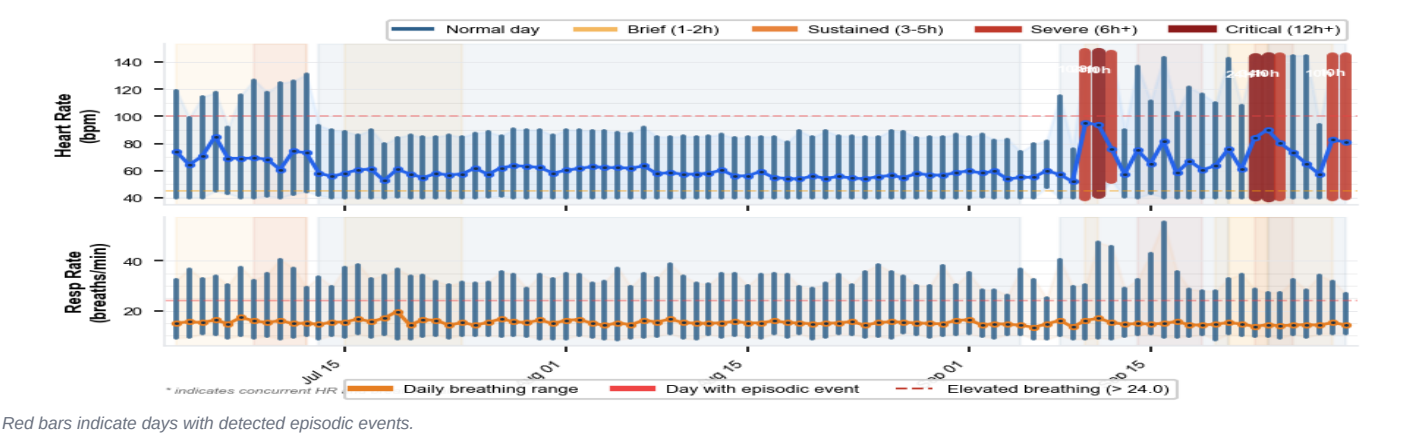
Suggested Clinical Review Actions

- Low Heart Rate (avg 43 bpm, min 40 bpm): Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
- Very High Heart Rate (avg 137 bpm, peak 145 bpm): Check temp, hydration, pain; review for arrhythmia or infection
- Elevated Heart Rate (avg 98 bpm, peak 131 bpm): Check temp, hydration, pain; review for arrhythmia or infection
- High Heart Rate (avg 105 bpm, peak 130 bpm): Check temp, hydration, pain; review for arrhythmia or infection

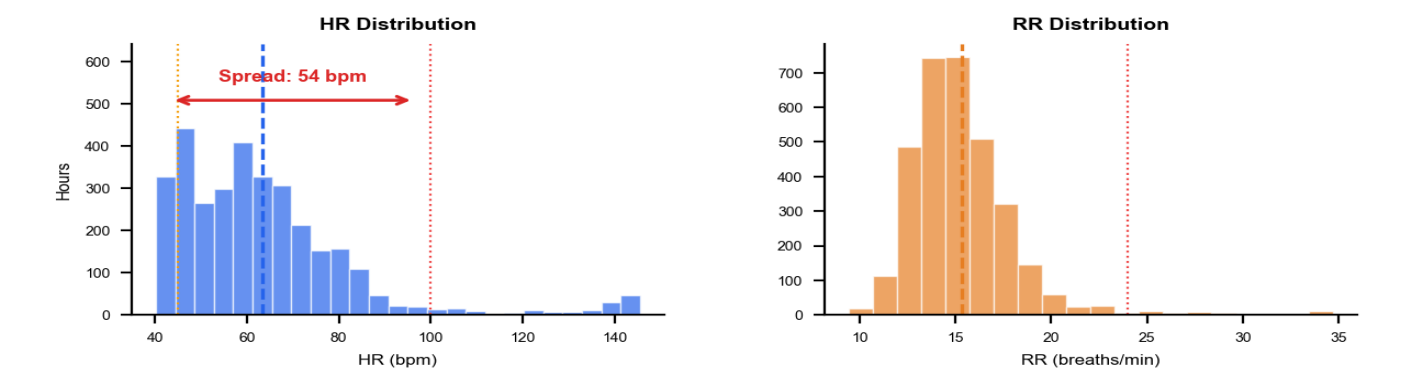
- Elevated Breathing (avg 27 brpm, peak 36 brpm): Check SpO2 and lung sounds; assess for fluid overload or early infection

Clinical Pattern Observations:

- 1. **Sustained finding:** 24h continuous Very High Heart Rate beginning Sep 23.
- 2. **High variability:** HR spread 54 bpm (42 to 96).
- 3. **Clustered pattern:** Episodic events on 8 of 91 days (8%).



Vital Sign Distribution



Monitoring Activity

